

Department of Obstetrics And Gynaecology
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 32.9 Yr/F
Admission No.: anonymous	Department Unit: Obstetrics And Gynaecology (Obs Unit 1)	Consultant/Faculty: anonymous
Ward: 1 B 2 Gynecology Ward	Room/Bed: 1 B 2 Gynecology Ward/ 1 B 2 Bed 2	Patient Category: Ayushman
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation:
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 15-Mar-2023 18:16	D.O.D. : 08-Feb-2026	D.O. Print Report: 08-Feb-2026 15:40

PROVISIONAL DIAGNOSIS:

30YR/P4L2SB1D1/PREV 2 VD/F/B 2 LSCS/ TL DONE/LCB 7 YR BACK/ ? CA OVARY 17 X 13.6 CM COMPLEX CYSTIC LESION IN PELVIS / B/L PLEURAL EFFUSION/ ASCITES

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FINAL DIAGNOSIS:

30YR/P4L2SB1D1/PREV 2 VD/F/B 2 LSCS/ TL DONE/LCB 7 YR/ POD 6 OF STAGING LAPAROTOMY WITH LEFT OVARIAN CYSTECTOMY WITH FROZEN SECTION WITH RIGHT CYSTECTOMY WITH OMENTAL BIOPSY WITH APPENDICECTOMY

30YR/P4L2SB1D1/PREV 2 VD/F/B 2 LSCS/ TL DONE/LCB 7 YR/ POD 6 OF STAGING LAPAROTOMY WITH LEFT OVARIAN CYSTECTOMY WITH FROZEN SECTION WITH RIGHT CYSTECTOMY WITH OMENTAL BIOPSY WITH APPENDICECTOMY

OPERATION PERFORMED:

STAGING LAPAROTOMY WITH LEFT OVARIAN CYSTECTOMY WITH FROZEN SECTION WITH RIGHT CYSTECTOMY WITH OMENTAL BIOPSY AND APPENDICECTOMY On 30-Mar-2023

SURGEONS: Dr Sarita Agrawal

ASSISTANT SURGEONS/SENIOR RESIDENTS: Dr Vijayalakshmi, Dr Akriti

OPERATIVE FINDINGS: UNDER AAP,PATIENT IN DORSAL POSITION.ALL PARTS ,PREPARED,PAINTED AND DRAPED.INFRAUMBILICAL MIDLINE VERTICAL INCISION GIVEN.ABDOMEN OPENED IN LAYERS.200cc PALE YELLOW MUCINOUS FLUID IN THE PERITONEAL CAVITY.10cc OF PERITONEAL FLUID SENT FOR CYTOLOGY.15X14 cm CYSTIC MASS SEEN IN THE ABDOMINAL CAVITY WITH OMENTAL ADHESION OVER THE CYSTIC WALL.ADHESIOYSIS DONE AND OMENTUM SEPARATED CAREFULLY FROM THE CYSTIC WALL AND CUTTING THE OMENTUM.CYST EXTERIORISED AND IS FOUND ARISING FROM RIGHT OVARY.RIGHT OVARY WITH CYST CLAMPED,CUT AND TRANSFIXED AND SENT FOR FROZEN SECTION.5X4 cm SIMPLE CYST IN LEFT OVARY.LEFT OVARIAN CYSTECTOMY DONE BY CLAMPING,CUTTING AND TRANSFIXING.DENSE OMENTAL ADHESIONS OVER THE ANTERIOR SURFACE OF UTERUS.ADHESIOYSIS DONE.FROZEN SECTION REPORT CAME OUT TO BE BENIGN MUCINOUS CYSTADENOMA.PALPATION OF ABDOMINAL VISCERA DONE IN CLOCKWISE DIRECTION STARTING FROM CAECUM,ACENDING COLON,HEPATIC FLEXURE OF COLON,UNDERSURFACE OF LIVER AND GALLBLADDER,TRANSVERSE COLON,RIGHT AND LEFT HEMIDIAPHRAGM,SPLENIC FLEXURE OF COLON,SPLEEN,DESCENDING COLON AND SIGMOID COLON.ALL WERE FOUND TO BE NORMAL.OMENTAL BIOPSY TAKEN AND APPENDICECTOMY DONE AND BOTH SMAPLES SENT FOR HPE.SALINE WASH GIVEN WITH 1.5 L OF LUKEWARM SALINE.HEMOSTASIS ACHIEVED.BLADDER DRAINS CLEAR URINE.ABDOMEN CLOSED IN LAYERS.PROCEDURE UNEVENTFUL.PATIENT WITHSTOOD THE PROCEDURE WELL.



CHIEF COMPLAINTS:

ABDOMINAL DISTENTION * 15 DAYS

COUGH* 15 DAYS

BREATHLESSNESS*10DAYS

NOT ABLE TO EAT/DRINK PROPERLY*10 DAYS

HISTORY OF PRESENT ILLNESS: 30YR/P4L2SB1D1/PREV 2 VD/F/B 2 LSCS/ TL DONE/LCB 7 YRS CAME WITH C/O ABDOMINAL DISTENTION , COUGHING , BREATHLESSNESS SINCE 15 DAYS. ON EXAMINATION, BREATH SOUNDS WERE REDUCED ON RIGHT SIDE AND ABDOMEN GROSSLY DISTENDED. USG DONE OUTSIDE 20/02/23 WAS SUGGESTIVE OF LARGE CYSTIC LESION IN PELVIS (17 X 13.6CM) (SUSPICION OF OVARIAN NEOPLASIA). HENCE SHE WAS ADMITTED FOR FURTHER MANAGEMENT.

GENERAL EXAMINATION:

PR: 80MIN, BP: 100/70MM HG, RR: 18/MIN, TEMP: AFEBRILE

SPO2 96 % ON ROOM AIR , 98 % ON 2 L O2

PALLOR /ICTERUS/CYANOSIS/CLUBBING/EDEMA: ABSENT

SYSTEMIC EXAMINATION:

CNS-CONSCIOUS, ORIENTED

CVS-S1 AND S2 REGULAR, NO MURMUR

RS- LEFT SIDE A/E , RIGHT SIDE REDUCED A/E ,NO ADDED SOUND

P/A: ABDOMEN GROSSLY DISTENDED . NOT VERY TENSED , NT , SHIFTING DULLNESS ,FLUID THRILL

P/S – CX- CENTRAL,SLIGHTLY DEVIATED TO RIGHT OS PINPOINT,SMOOTH REGULAR.PROFUSE CURDY WHITE D/C , VULVA VAGINA HEALTHY .

P/V- CX- CENTRAL,SLIGHTLY DEVIATED TO RIGHT SIDE ,SMOOTH REGULAR , UTERUS NORMAL SIZE, A/V , NT , POD – FREE, NT . B/L FORNICES NOT ASSESSED D/T TENSE ASCITIS

INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
31-Mar-2023 10:12	COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.02	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.44	0.02 - 0.5 x10 ³ /μL
		Absolute Lymphocyte Count	1.25	1 - 3 x10 ³ /μL
		Absolute Monocyte Count	0.75	0.2 - 1.0 x10 ³ /μL
		Absolute Neutrophil Count	9.01	2 - 7 x10 ³ /μL
		Basophil	0.2	00 - 01 %
		Eosinophils	3.8	01 - 06 %
		HCT	29.0	36 - 46 %
		Hemoglobin	8.8	12 - 15 g/dl
		Lymphocytes	10.9	20 - 40 %
		MCH	24.3	27 - 32 pg
		MCHC	30.3	31.5 - 34.5 g/dL
		MCV	80.1	83 - 101 fL
		Monocytes	6.5	2 - 10 %

		Neutrophils	78.6	40 - 80 %
		Platelet Count	304	150 - 410 x10 ³ /μL
		RBC	3.62	3.8 - 4.8 x10 ⁶ /μL
		Red Cell Distribution Width (RDW CV)	13.2	11.6 - 14.0 %
		Total Leukocyte Count(TLC)	11.47	4 - 10 x10 ³ /μL
30-Mar-2023 16:49	EXFOLIATIVE CYTOLOGY	Result	Refer Reports*	-
28-Mar-2023 10:02	ACTIVATED PARTIAL THROMBOPLASTIN TIME (APTT)	APTT HEADER	26.2	23.2 - 32.4 sec
		APTT NOTE	Refer Reports*	-
		clinical details	--	-
	COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.03	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	2.48	0.02 - 0.5 x10 ³ /μL
		Absolute Lymphocyte Count	2.03	1 - 3 x10 ³ /μL
		Absolute Monocyte Count	0.25	0.2 - 1.0 x10 ³ /μL
		Absolute Neutrophil Count	3.93	2 - 7 x10 ³ /μL
		Basophil	0.3	00 - 01 %
		Eosinophils	28.4	01 - 06 %
		HCT	39.4	36 - 46 %
		Hemoglobin	11.5	12 - 15 g/dl
		Lymphocytes	23.3	20 - 40 %
		MCH	23.8	27 - 32 pg
		MCHC	29.2	31.5 - 34.5 g/dL
		MCV	81.4	83 - 101 fL
		Monocytes	2.9	2 - 10 %
		Neutrophils	45.1	40 - 80 %
		Platelet Count	252	150 - 410 x10 ³ /μL
		RBC	4.84	3.8 - 4.8 x10 ⁶ /μL
		Red Cell Distribution Width (RDW CV)	12.6	11.6 - 14.0 %
		Total Leukocyte Count(TLC)	8.72	4 - 10 x10 ³ /μL
	PROTHROMBIN TIME (PT INR)	clinical details	--	-
		International Normalised Ratio (INR)	1.1	-
		PT INR Header	10.4	09.6 - 12.2 sec

		PT INR Note	Refer Reports*	-
24-Mar-2023 09:43	ERYTHROCYTE	ESR	Refer Reports*	-
	SEDIMENTATION			
	RATE (ESR)			
17-Mar-2023 15:39	Bacterial Culture	Colony Count	--	-
		Comments	--	-
		Culture Method	Aerobic	-
		Culture Result	No bacterial growth obtained after 48 hrs. of aerobic incubation	-
		Culture Result Positive	-1	-
		Incubation Period	48 hrs	-
		Interpretation		-
		Microscopy	-1	-
		Microscopy Result	--	-
		Note	--	-
		Other Culture	--	-
		Specimen	Ascitic fluid	-
	BODY FLUID ROUTINE	Atypical cells	absent	-
	AND MICROSCOPIC			
	EXAMINATION			
		Basophils	00	-
		Blood	absent	-
		Coagulum	absent	-
		Colour	yellow	-
		Comments	Refer Reports*	-
		Degenerated cells	absent	-
		Deposit	absent	-
		Eosinophils	00	-
		Lymphocytes	100	-
		Mesothelial cells	absent	-
		Monocytes	00	-
		Neutrophils	00	-
		Others	absent	-
		Specimnn	Peritoneal Fluid	-
		Total Leucocyte Count	10	-
		Turbidity	turbid	-
		Volume	15	-
	CBNAAT GeneXpert	Comments	Specimen - Pleural fluid	-
		Foot Notes	Refer Reports*	-
		Mycobacterium Tuberculosis	NOT DETECTED	-
		Rifampicin Resistance	NOT APPLICABLE	-

		Test Performance	VALID	-
	EXFOLIATIVE	Result	Refer Reports*	-
	CYTOLOGY			
17-Mar-2023 08:03	AFP	AFP	less than 1.3	0 - 8 ng/ml
16-Mar-2023 19:41	CA125	CA 125	72.6	< 30.2 U/ml
	CA 19.9	CA 19.9	greater than 700.00	< 30.9 U/ml
	CEA	CEA	less than 0.50	0 - 2.5 ng/ml
	THCG	Result	Refer Reports*	-
		THCG	less than 2.0	-
16-Mar-2023 15:10	BODY FLUID ROUTINE AND MICROSCOPIC EXAMINATION	Atypical cells	Absent	-
		Basophils	00	-
		Blood	Absent	-
		Coagulum	Absent	-
		Colour	Pale yellow	-
		Comments	Refer Reports*	-
		Degenerated cells	Absent	-
		Deposit	Absent	-
		Eosinophils	00	-
		Lymphocytes	30	-
		Mesothelial cells	Present	-
		Monocytes	00	-
		Neutrophils	70	-
		Others	RBCs seen	-
		Specimnn	Pleural Fluid	-
		Total Leucocyte Count	100	-
		Turbidity	Clear	-
		Volume	10	-
	CBNAAT GeneXpert	Comments	Specimen - Ascitic fluid	-
		Foot Notes	Refer Reports*	-
		Mycobacterium Tuberculosis	NOT DETECTED	-
		Rifampicin Resistance	NOT APPLICABLE	-
		Test Performance	VALID	-
	EXFOLIATIVE	Result	Refer Reports*	-
	CYTOLOGY			
	PLEURAL FLUID ALBUMIN	PLEURAL FLUID ALBUMIN	2.1	--- gm/dl
	PLEURAL FLUID LDH	PLEURAL FLUID LDH	104	--- U/L
	PLEURAL FLUID PROTEIN	PLEURAL FLUID PROTEIN	3.6	--- gm/dl



16-Mar-2023 11:27	FASTING GLUCOSE	Fasting Blood Sugar	86	70 - 110 mg/dl
	Liquid Based Cytology	Comment	Refer Reports*	-
		Result	Refer Reports*	-
	PP GLUCOSE	Glucose P P	81	< 140 mg/dl
	FT4	FT4	1.64	0.9 - 1.7 ng/dL
	SICKLING TEST	ST Header	Refer Reports*	-
		ST Note	Refer Reports*	-
	TSH UL	TSH UL	2.825	0.55 - 4.78 mIU/L
	URINE ROUTINE AND MICROSCOPIC EXAMINATION (R/M)	Appearance	SLIGHTLY TURBID	-
		Bilirubin	NOT DETECTED	-
		Casts	NOT DETECTED	-
		Colour	DARK YELLOW	-
		Comments	Refer Reports*	-
		Crystals	CALCIUM OXALATE	-
		Epithelial Cells	NOT DETECTED	0 - 5 / hpf
		Glucose	NOT DETECTED	-
		Ketones	NOT DETECTED	-
		Nitrite	NEGATIVE	-
		Others	NOT DETECTED	-
		Protein		-
		Pus cells	20-50	-
		RBC	NOT DETECTED	-
		Reaction pH	6.0	5 - 7 .
		Specific Gravity	1.015	1.003 - 1.035 .
		Urobilinogen	NORMAL	-
15-Mar-2023 21:39	HBsAg (ELISA)	Interpretation	Non Reactive	-
		Result	Refer Reports*	-
	HCV Antibody (ELISA)	Interpretation	Non Reactive	-
		Result	Refer Reports*	-
	LACTATE DEHYDROGENASE (LDH)	LDH	243	230 - 460 u/l
15-Mar-2023 20:07	ACTIVATED PARTIAL THROMBOPLASTIN TIME (APTT)	APTT HEADER	26.6	23.2 - 32.4 sec
		APTT NOTE	Refer Reports*	-
		clinical details	--	-
	BLOOD GROUPING ABO and RhD	Blood Group	O	-

	Note	Refer Reports*	-
	RH Type	Positive	-
COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
	Absolute Basophil Count	0.06	0.02 - 0.10 x10 ³ /μL
	Absolute Eosinophil Count	0.78	0.02 - 0.5 x10 ³ /μL
	Absolute Lymphocyte Count	2.54	1 - 3 x10 ³ /μL
	Absolute Monocyte Count	0.74	0.2 - 1.0 x10 ³ /μL
	Absolute Neutrophil Count	4.72	2 - 7 x10 ³ /μL
	Basophil	0.7	00 - 01 %
	Eosinophils	8.8	01 - 06 %
	HCT	42.4	36 - 46 %
	Hemoglobin	12.5	12 - 15 g/dl
	Lymphocytes	28.7	20 - 40 %
	MCH	23.9	27 - 32 pg
	MCHC	29.5	31.5 - 34.5 g/dL
	MCV	81.1	83 - 101 fL
	Monocytes	8.4	2 - 10 %
	Neutrophils	53.4	40 - 80 %
	Platelet Count	413	150 - 410 x10 ³ /μL
	RBC	5.23	3.8 - 4.8 x10 ⁶ /μL
	Red Cell Distribution Width (RDW CV)	13.1	11.6 - 14.0 %
	Total Leukocyte Count(TLC)	8.84	4 - 10 x10 ³ /μL
Liver Fuction Test (Lft)	Albumin	3.5	4.0 - 5.0 gm/dL
	ALP	75	46 - 122 U/L
	ALT	11	7 - 41 U/L
	AST	20	12 - 38 U/L
	Direct Bilirubin	0.14	0.1 - 0.4 mg/dl
	globulin	3.3	2.0 - 3.5 gm/dL
	Indirect Bilirubin	0.40	0.20 - 0.90 mg/dl
	Total Bilirubin	0.54	0.3 - 1.3 mg/dl
	Total Protein	6.8	6.7 - 8.6 gm/dL
PROTHROMBIN TIME (PT INR)	clinical details	--	-
	International Normalised Ratio (INR)	1.1	-
	PT INR Header	11.0	09.6 - 12.2 sec
	PT INR Note	Refer Reports*	-
Rft	Chloride	101	102 - 109 mmol/L



Creatinine	0.89	0.6 - 1.1 mg/dl
Potassium	3.87	3.5 - 5 mmol/L
Sodium	137	135 - 145 mmol/L
Urea	18	15.0 - 39.0 mg/dl

***Refer Lab Reports for details.**

OTHER INVESTIGATIONS: BLOOD GROUP O POSITIVE

LBC (16/03/23) ; INFLAMMATORY SMEAR

USG (20/2/23)-LARGE MULTILOCULATED ,MULTISEPTATED ,OVERECHOIC CYSTIC LESION OF SIZE 17X13.6CM SEEN ARISING FROM PELVIS ,EXTENDING IN LOWER ABDOMEN , B/L OVARIES ARE NOT VISUALISED WELL . FEW SOLID COMPONENTS SEEN WITHIN LIKELY NEOPLASTIC ETIOLOGY OF OVARIAN ORIGIN . MODRATE TO GROSS ASCITIS IN ABD AND PELVIS . B/L PLEURAL EFFUSION.

PLEURAL FLUID CBNAAT - M.TB NOT DETECTED

ASCITIC FLUID CBNAAT - M.TB NOT DETECTED

PERITONEAL CYTOLOGY - NEGATIVE FOR MALIGNANCY

PETCT (22/3/23) : NON TO FAINT FDG AVID LARGE, LOCULATED, MULTISEPTATED SOLID CYSTIC DENSITY LESION (12.6 X 12.9 X 14.7CM) IN ABDOMINO PELVIC CAVITY LIKELY ARISING FROM LEFT ADNEXA. GROSS ASCITIS PRESENT.

TUMOUR MARKERS : (17/03/23)

AFP <1.3

CA 19.9 > 700 U/ML (HIGHLY RAISED)

CEA < 0.50 NG/ML

CA 125 72.6 U/ML (MILDLY RAISED)

THCG < 0.20

LDH 243 U/L

HOSPITAL COURSE: 30YR/P4L2SB1D1/PREV 2 VD/F/B 2 LSCS/ TL DONE/LCB 7 YRS CAME WITH C/O ABDOMINAL DISTENTION , COUGHING , BREATHLESSNESS SINCE 15 DAYS. ON EXAMINATION, BREATH SOUNDS WERE REDUCED ON RIGHT SIDE AND ABDOMEN GROSSLY DISTENDED. USG DONE OUTSIDE 20/02/23 WAS SUGGESTIVE OF LARGE CYSTIC LESION IN PELVIS (17 X 13.6CM) (SUSPICION OF OVARIAN NEOPLASIA). HENCE SHE WAS ADMITTED FOR FURTHER MANAGEMENT. GENERAL MEDICINE OPINION WAS TAKEN IN VIEW OF BILATERAL PLEURAL EFFUSION AND PLEURAL FLUID AND ASCITIC TAPPING ADVISED. PULMONARY MEDICINE OPINION WAS TAKEN AND ICD DRAINAGE DONE ON 16/03/23 AND PLEURAL FLUID SENT FOR ALL RELEVANT INVESTIGATIONS. POST ICD INSERTION REVIEW PULMONARY MEDICINE OPINION WAS TAKEN AND ADVISES FOLLOWED. ASCITIC TAPPING UNDER USG GUIDANCE WAS DONE ON 17/03/23 AND ASCITIC FLUID SENT FOR ALL RELEVANT INVESTIGATIONS. ALL TUMUR MARKERS WERE SENT AND WERE WITHIN NORMAL LIMITS EXCEPT CA 19.9 (> 700 U/ML - HIGHLY RAISED) AND CA 125 (72.6 U/ML - MILDLY RAISED). CARDIOLOGY OPINION WAS TAKEN IN VIEW OF T WAVE INVERSIONS ON ECG, 2D ECHO DONE, WAS WITHIN NORMAL LIMITS, SURGICAL FITNESS GIVEN FROM THE CARDIOLOGY SIDE. PETCT WAS DONE ON 21/3/23 WHICH WAS SUGGESTIVE OF NON TO FAINT AVID LARGE LOCULATED SOLID CYSTIC LESION MEASURING 12.6 X 12.9 X 14.7 CM ARISING FROM LEFT ADNEXA WITH GROSS ASCITES. PULMONARY MEDICINE OPINION WAS TAKEN ON REGULAR BASIS REGARDING ICD DRAIN OUTPUT, IT WAS REMOVED ON 23/3/23 AS ICD DRAIN OUTPUT HAD REDUCED AND LUNGS HAD EXPANDED COMPLETELY. MONTOUX TEST WAS PERFORMED ON 24/3/23 WHICH TURNED OUT POSITIVE. PAC CLEARANCE WAS TAKEN. SHE UNDERWENT STAGING LAPAROTOMY WITH LEFT OVARIAN CYSTECTOMY WITH FROZEN SECTION WITH RIGHT CYSTECTOMY WITH OMENTAL BIOPSY WITH APPENDICECTOMY ON 30/03/23. FROZEN SECTION REPORT WAS SUGGESTIVE OF BEINGN MUCINOUS CYSTADENOMA . THERE WERE NO INTRAOP OR POSTOP COMPLICATIONS. PATIENT PASSED STOOL AND URINE. POD 3 CHECK DRESSING WAS DONE, STITCH SITE HEALTHY. PATIENT CONDITION IS FAIR NOW AND HENCE BEING DISCHARGED WITH ADVISE TO FOLLOW UP IN GYNAEC OPD WITH HPE REPORT FOR



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91-0120-3063311-13.



SUTURE REMOVAL.

CONDITION AT DISCHARGE: GC- FAIR, AFEBRILE

HR: 82MIN, BP:110/70 MM HG,RR- 18/MIN

CNS/RS/CVS: WNL

P/A: SOFT,NON TENDER, STITCH LINE HEALTHY

LOCAL EXAMINATION : .NO ACTIVE BLEEDING

ADVICE ON DISCHARGE:

HIGH PROTEIN DIET.

TAB CEFIXIME 200MG BD 1 DAY

TAB METRO 400MG TDS FOR 1 DAY

TAB PANTOP 40 MG OD FOR 1 DAY

TAB PCM 500MG TDS FOR 1 DAY F/B SOS IF PAIN

TAB ONDEN 5 MG SOS IF VOMITING

TAB CALCIUM VITAMIN D3 BD X 3 MONTHS

TAB IRON FOLIC ACID 1 TAB PO OD FOR 3 MONTHS

TAB VITAMIN C 500MG 1 TAB PO OD FOR 3 MONTHS

TAB MVBC 1 TAB OD FOR 3 MONTHS

SYP LACTULOSE 15ml HS IF HARD STOOLS

BATH REGULARLY

WASH STITCH SITE WITH SOAP WATER AND KEEP DRY

METRO P OINTMENT LOCAL APPLICATION BD

SEXUAL ABSTINENCE 6 WEEKS

DISCHARGE BY : anonymous

DISCHARGE REMARKS :

FOLLOW UP ON : 10-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE : FOLLOW UP IN GYNAEC OPD ON 10/04/23 FOR SUTURE REMOVAL WITH HPE REPORT OR SOS IN CASE OF SEVERE PAIN ABDOMEN, BLEEDING PV, DISCHARGE FROM STITCH SITE OR FEVER.

Summary Prepared By

anonymous

Summary Approved By

anonymous